FICTIONAL - FOR DEMO

Riverside Community Hospital - Discharge Summary

Patient: Riley Sanders (fictional), 67-year-old adult
Admission date: November 1, 2026
Discharge date: November 5, 2026
Attending: Dr. Henry Zhao, Pulmonary Medicine

Diagnosis:
COPD exacerbation (flare-up).

Hospital course:
You came in with worsening shortness of breath, more cough, and more mucus than usual.
You received steroids, breathing treatments, and oxygen.
Your breathing has improved and you no longer need oxygen at rest.

Medications:
Prednisone 40 mg by mouth once daily with breakfast for 5 more days.
Then stop.
Do not stop it early.
Albuterol inhaler (Ventolin HFA) 90 mcg per puff, 2 puffs inhaled every 4 to 6 hours as needed, ongoing.
Use it for wheezing or shortness of breath.
Tiotropium (Spiriva HandiHaler) 18 mcg, 1 capsule inhaled once daily, ongoing.
Inhale the capsule powder with the HandiHaler device.
Do not swallow the capsule.

Diet:
Eat smaller meals more often if you feel short of breath when full.
Drink fluids to keep mucus thin unless told otherwise.

Activity:
Walk a little more each day, resting when you need to.
Try pursed-lip breathing during activity.
Do not smoke.
Avoid smoke, strong fumes, and very cold air.

Inhaler care:
Rinse your mouth with water after using the Spiriva HandiHaler.
Keep a list of how many puffs of albuterol you use each day.

Monitoring:
Count how often you use your albuterol inhaler each day for 7 days and bring the list to your visit.
Check your temperature every evening for 7 days.

Follow-up:
See Dr. Henry Zhao, Pulmonary Clinic on November 12, 2026 for a flare-up follow-up and an inhaler technique check.
See Pulmonary rehabilitation on November 26, 2026 for an intake visit for a breathing exercise program.

Call 911 or return to the ER if:
- severe trouble breathing or you cannot speak in full sentences
- chest pain
- blue or gray lips or fingertips
- confusion or extreme sleepiness
- fainting

Call the clinic at 555-0113 if:
- you need albuterol more often than every 4 hours
- your mucus turns green or bloody
- a fever of 101 F (38.3 C) or higher

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